



Deposit/Advance Receipt

IPNO : 252053
Patient Name : Mr. VIPAN KUMAR GARG
Patient Address : HNO 323 FF, SEC P PANCHKULA HR Panchkula
Panchkula Industrial Area Phase 1, Haryana, 134113
India
Doctor : Sachin Gupta
Bed : MO-OCDC-14

UHID
Receipt No.
Receipt Date
AdmitDateTime
GSTN No
GSTSeriesNo.
Place Of Supply
PAN No.

HDFC BANK
We understand your world

HDFC BANK
MAX SUPER SPECIALITY HOSPITAL PANCHKULA
MOHALI
DATE: 2025-09-23
TIME: 13:28:05
BASE AMT.: INR 40189.00
PIN VERIFIED OK
SIGNATURE NOT REQUIRED
GARG/VIPAN KUMAR
I AGREE TO PAY AS PER CARD ISSUER AGREEMENT
**** CUSTOMER COPY ****
Plutus v1.49.3 MT HDFC
<https://www.pinelabs.com/privacy-policy> to APPLY

pine labs

Sl.No.	SACCODE	Service Name	Mode of payment	
1	999311	Inpatient services	CREDIT CARD	40,189.00
Total Amount (Rs) :				40,189.00

Credit Card : XXXXXXXXXXXX4541
Approval Number : 120, Tr. Id : 572080861, Tr. dt. : 23-09-2025 13:28
Bank Name : HDFC

Amount In Words : Rupees forty thousand one hundred eighty-nine only
Received With thanks from Mr. VIPAN KUMAR GARG
on account of IP No. 252053

SGST@ 0.00% : 0.00
CGST@ 0.00% : 0.00
UTGST@ 0.00% : 0.00

Deepak Kumar

Cashier

25
YEARS OF
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T: +91-70424 01894
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(CIN: U45400DL2008PTC176962)



MC-2287

Vipon Kumar
435714

Please follow up in Oncodaycare with Dr. Sachin Gupta/Dr. Krutika Goel on 21.10.2025 with CBC, KFT, LFT & RBS for chemotherapy.

Inj Denosumab 120mg s/c stat due on November-2025.

-Serum free light chain assay & Serum protein electrophoresis due in November-2025.

For Scheduling Appointments please contact: Call Centre (0172-5212000 Or Please visit the front-office to get your next appointment booked.

Do not call Doctors mobile for appointment)- Call Appointment desk. Between 9 am to 5 Pm Call in Oncology Day Care for any Emergency/Or any otherquery (2856)

In Emergency please contact: 2666 / 77107-77107(Max Hospital Emergency No) Dr. Sachin Gupta (6280334490), Dr Krutika 7988689148

To Send Email to Shama@maxhealthcare.com and Whatsapp: 7973934490 (9:00 AM to 6:00 PM)

I have understood and have been given an opportunity to clarify my concerns in the language I/we understand & hereby acknowledge receipt of the discharge summary.

Patient Name : Vipon Kumar Garg

Patient/Attendant

Nurse

IN CASE OF EMERGENCY :-

/es/ Ladi Saluja
Medical Transcriptionist - L1

/es/ Ladi Saluja
Medical Transcriptionist - L1
Cosigned: 23 SEP, 2025 11:57
for Sachin Gupta
Director

Entered Date : 23 SEP, 2025 10:32

Prepared By: Ladi Saluja

SSN No. : 150435714

For post-hospital care at home, call Max@Home at 8744 888 888 (24x7 helpline). Services include Critical Care@home Nursing Healthcare Attendant, Physio-therapy, X-Ray, Sample Collection, Medicine Delivery, Medical Equipment and more.

IN CASE OF ANY EMERGENCY PLEASE CONTACT 0172-5212000.

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May 28, 19 - May 27, 22
Since May 28, 2013

Vipin Kumar
435714

Syp Cremaffin Plus 15ML at bed time for constipation x 3 days.

K-Bind sachet, one sachet thrice daily in full glass of water x 3 days.

Foracort puff 2 puff at bed time

Hexidine mouthwash 10 ml oral rinse three times daily to continue, preferably after meals.

Plenty of water

SOS LIST

Tab Normaxin 1 tablet SOS for abdominal cramps

Movicol Sachet 1 sachet twice daily / sos for constipation

Tab Flexon 1 tablet SOS for body aches

Tab Dulcolax 2 tab sos for constipation

Tab Dolo 650 mg 1 tablet SOS for pain/fever

Syp Cremaffin 10 ml SOS at bed time for constipation

Special Needs:

Special Precautions

If you experience any of the following, report to emergency room at Max Super

1. Severe weakness /Severe mouth ulcers
2. Fever
3. Loose stools/motions/vomitings
4. Bleeding from any site or passing black stools like coal tar.
5. Chest pain, breathing difficulty, pain in abdomen.
6. Reduced urine output.
7. Rash over skin, swelling over body
8. Any other problem for which you think urgent attention is required.

Follow Up Advice:

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MC-2287

IN-2013-0181
May 28, 19 - May 22, 22
Since May 28, 2013

Vipin Kumar
435714

21.08.2025

Free Kappa 38.71
Free Lambda 16.58
Kappa/Lambda, Ratio 2.335

Discharge Medication Advice:

- Tab Pomalidomide 2 mg one tablet once daily 3 weeks on and 1 week off
- Tab Pantop 40 mg 1 tablet once daily (Before Breakfast) x 3 days then SOS for acidity
- Tab Emeset 8 mg 1 tablet three times daily x 3 days then SOS for Nausea/Vomiting
- Tab Ecosprin 75 one tablet at bed time to continue.
- Tab Shelcal one tablet once daily to continue.
- Tab Acivir 400mg one tablet twice daily to continue.
- Tab Thyronorm 50 mcg 1 tablet once daily to continue
- Tab Torsemid 20 mg one tablet once daily before breakfast to continue.
- Tab Nicardia XL 30 mg one tablet twice daily before breakfast and before dinner to continue.
- Tab Arkamin 0.1 mg one tablet thrice daily after meals to continue.
- Tab Metoprolol SR 50 mg one tablet after breakfast to continue.
- Tab Silodosin D8 one tablet at bed time to continue.
- Tab Irobor LP one tablet before lunch to continue.
- Tab Atarax 25 mg one tablet once daily to continue.
- Tab Auxisoda EC 1000mg one tablet twice daily before breakfast and before dinner to continue.
- Tab Dytor 10 mg one tablet SOS

Patient Name : Vipin Kumar Garg
SSN No. : 150435714

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MC-2287

14-2015-0181
May 28, 15 - May 27, 22
Since May 28, 2013

Vipin Kumar
435714

IgG -6.84
IgM-< 21.0
IgA-< 33.0

29.11.2024

Free Kappa 20.31
Free Lambda 7.25
Kappa/Lambda, Ratio 2.801

Protein Electrophoresis 24 hour urine
Total Protein - 0.14
Total Urine Volume - 2400

10.02.2025

Free Kappa 46.57
Free Lambda 15.04
Kappa/Lambda, Ratio 3.096

SERUM PROTEIN IFE ELECTROPHORESIS 10.02.2025

M Spike - ? 0.2

Suspicious non-discrete protein band in gamma globulin region, possibly monoclonal gammopathy.

24.05.2025

Free Kappa 48.70
Free Lambda 19.25
Kappa/Lambda, Ratio 2.530

SERUM PROTEIN IFE ELECTROPHORESIS 24.05.2025

M Spike - ? 0.2

Suspicious non-discrete protein band in gamma globulin region, possibly monoclonal gammopathy.

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Vipau Kumar
4305714

No Lymphadenopathy

Past Investigation :

MOLECULAR/ PREDICTIVE/ TUMOR MARKERS

02.05.2024
Free Kappa 20.82
Free Lambda 11.80
Kappa/Lambda, Ratio 1.7600

SERUM PROTEIN IFE ELECTROPHORESIS 02.05.2024

"M" Spike seen in gamma globulin region is of
IgG kappa type.

SERUM PROTEIN ELECTROPHORESIS 03.05.2024

Hypoproteinemia, Hypoalbuminemia and hypogammaglobulinemia present.
Monoclonal gammopathy ("M" Spike 0.17 g/dl) seen in gamma globulin region.
(In a follow - up case).

22.07.2024

Free Kappa 18.66
Free Lambda 17.14
Kappa/Lambda, Ratio 1.089

SERUM PROTEIN IFE ELECTROPHORESIS 22.07.2024

Hypoproteinemia present
No "M" spike seen.
Hypogammaglobulinemia present.

29.11.2024

Serum protein profile by high-resolution Agarose gel electrophoresis reveals the presence of a faint non-discrete protein band in gamma globulin region, which could be 'M' spike.
Immunofixation electrophoresis (IFE) identifies the very faint non-discrete protein band (?) in gamma globulin IgG and Kappa lane corresponding to suspicious 'M' spike.

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Vipau Kumar
455714

Patient was on Maintenance Daratumumab + Tab Pomalidomide from Jan 2025.

Plan:-

Inj Daratumumab 1200 mg D1Q monthly
Inj Denosumab 120 mg s/c stat3 monthly
Cap Pomalidomide 2 mg 1 cap 3 weeks on & 1 week off

Presenting Complaints:

Admitted for chemotherapy

Past Medical History:

Comorbidities

Hypertension
Hypothyroidism
H/O Bronchial Asthma since 20 years

Course in Hospital:

Chemotherapy Cycle 21 Day 1 on 23.09.2025:- (Maintenance Daratumumab)

Inj Daratumumab 1200 mg s/c
with premedication

Patient tolerated chemotherapy well and is being discharged with below mentioned medication advise---

On Examination:

Pallor - Icterus -
CNS : Conscious, Oriented
CVS : S1S2 heard
Resp: Bilateral A entry equal
P/A : Soft, Non-tender, No Hepatosplenomegaly

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MC-2287
May 20, 19 - May 27, 22
Since May 20, 2013



Vipin Kumar
435714

Plan :-

Inj Carfilzomib 54mg/m² IV D1, D8, D15
Inj Daratuzumab 1200 mg IV D1
Tab Pomalidomide 2 mg PO x 1 month
Inj Zoledronic 4mg IV monthly

In Sept 2024:- Patient presented with the complaint of anuric status and raised creatinine level (5.54) in view of aki. Patient was already undergoing treatment from Medanta hospital, Gurgaon for the same. Patient was admitted in medanta hospital with complaint of fever and cough and later had reduced urine output and was later anuric. Onco medicine consult was done and hemolysis workup was sent which revealed indirect coombs test positive. Dialysis was done in view of fluid overload regularly.

He was also found to have severe thrombocytopenia and anemia over there. In view of the above, differential diagnosis of HUS was also kept. Kidney biopsy done as he wasnt improving and remained anuric. Renal biopsy report awaited. Gradually improvement in urine output - 400ml /day.

Streptococcus Pneumonia

Already resolved by the time he was shifted to Max, Mohali.

USG ABDOMEN WITH PELVIS OUTSIDED ON 08.09.2024

-BOTH KIDNEYS SHOW MILD DIFFUSE INCREASED RENAL PARENCHYMAL ECHOTEXTURE

ECHO OUTSIDED ON 09.09.2024

NORMAL LV SYSTOLIC FUNCTION (LVEF=65%)
TYPE I DIASTOLIC DYSFUNCTION

SPUTUM ANALYSIS OUTSIDED ON 08.09.2024

STREPTOCOCCUS PNEUMONIA-DETECTED (CT VALUE-30)

HRCT CHEST DONE OUTSIDED ON 07.09.2024

MINIMAL BILATERAL PLEURAL EFFUSION WITH FISSURAL EXTENSION,PASSIVE ATELECTASISAND SMOOTH INTERLOBULAR SEPTAL THICKENING OF THE UNDERLYING LUNG PARENCHYMA,FEW FOCAL AREAS OF AIRSPACE CONSOLIDATIONS ARE SEEN IN BASAL SEGMENTS OF BILATERAL L LOBES(RIGHT>LEFT)

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(CIN: U45400DL2008PTC176962)



May 28, 2024

Vipin Kumar
435714

Erythropoiesis: Normoblastic erythroid reaction
Myelopoiesis: Normal sequential maturation
Megakaryopoiesis: Megakaryocytes are adequate in number and normal in morphology.
Lymphocytes: Normal in number and morphology
Plasma cells: Mildly increased in number constituting 3% of total cellularity, including reactive forms.

BONE MARROW BIOPSY

Microscopic: Bone marrow biopsy is adequate for evaluation showing bony trabecular with 10 intertrabecular spaces,
Cellularity: Normocellular (60-65%)
Report-Bone marrow biopsy shows trilineage hematopoiesis with normal distribution of hematopoietic cells.
Plasma Cells: There is no appreciable increase in plasma cells OR focal clustering
Impression: Bone marrow appears to be in morphological remission in a follow-up case of multiple myeloma.

PET CT SCAN ON 13/02/24

IMPRESSION: Case of multiple myeloma, post chemotherapy with increase in kappa FLC during follow-up. PET/CT reveals:

1. Metabolically active lytic lesions involving few skeletal sites as described (pathological fractures involving right 6 & left 10 ribs, collapsed D1 vertebra) - residual/recurrent neoplastic disease involvement.
2. Non-metabolic lytic lesions in multiple other skeletal sites as described above; D1 & D2 vertebral collapse with no significant metabolic activity.
3. Also noted:
(i) Metabolically active multiple irregular fibronodules, few arranged in tree-in-bud pattern with adjacent ground glass haziness in the left lung with multiple mediastinal & right supraclavicular lymph nodes - favors active infective/inflammatory etiology.
(ii) Hepatomegaly.
(iii) Prostatomegaly.
4. No definite scan evidence of abnormal metabolically active disease in rest of the body in this study.

Previous FDG PET/CT scan CD not available for comparison (done at PGI, Chandigarh, dated 10.08.2023).

HRCT CHEST ON 22/03/24

Impression: K/c/o multiple myeloma, Present scan shows:

Multiple centrilobular nodules and areas of consolidation in left lower lobe-infective - Multiple lytic lesions throughout axial skeleton and right 6th rib with vertebra plana D6 with paravertebral soft tissue.
-Dilated thoracic esophagus with hiatal hernia

Then he was planned for chemotherapy with below mentioned plan in view of residual/recurrent neoplastic disease involvement.

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Page
May 28, 19 - 19
Since May

Vipin Kumar
435714SSN No. : 150435714
Patient Name : Vipin Kumar Garg

Investigations
Serum free light chain assay (30/05/2022)
Kappa - 857.95
Lambda - 17.82
Kappa/Lambda Ratio - 48.15

Urine Protein (01/06/2022) - 922 mg/TV

PBF (01/06/2022) : Mild anisocytosis. Normocytic normochromic red cells admixed with a few macrocytes. Platelets are adequate with a few large forms. No circulating plasma cells or Rouleaux formation seen.

MRI spine Dorsal (contrast)(25/07/2022):

-Multiple enhancing dorsal vertebral body and posterior element osseous lesions with pathological collapse D1 and D6 vertebral bodies -c/o MM .
-Vertebra plana D6 with enhancing bilateral para vertebral soft tissue component showing extradural intra spinal extension with bilateral foraminal compression and involvement of bilateral costochondral rib junctions .
-Minimal bilateral pleural effusion .

CT Thorax / Chest - HRCT Plain of 19-SEP-2022:

-Few tiny ground glass nodules are seen scattered in the left lower lobe. ?
infective (Adv follow up)
-Small lytic lesions in the multiple thoracic vertebral bodies with complete collapse of the D1 and D6 vertebrae.

Serum protein electrophoresis on 18/10/22 -Monoclonal gammopathy ("M" spike-0.06 g/dl) seen in gamma globulin region.

Serum protein electrophoresis on 16/02/23 -Monoclonal gammopathy ("M" spike-0.04 g/dl) seen in gamma globulin region

Serum protein electrophoresis on 23/05/2023 -no "M"spike seen

Serum protein electrophoresis on 30/12/23-Monoclonal gammopathy ("M" spike 0.24g/dl) seen in gamma globulin region

Received 4 cycles with Inj Bortezomib /Inj Daratumumab/ Inj Cyclophosphamide till 03.10.2022, Took Tab Endoxan 50 mg IN 2022.

BONE MARROW ASPIRATION AND BIOPSY ON 10/02/24

Peripheral Blood (Smear)

Findings: Peripheral smear shows normocytic, normochromic anemia with mild thrombocytopenia

BONE MARROW ASPIRATION and IMPRINT SMEAR REPORT (BM-35/24)

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MC-2287



14-2013-0181
May 28, 19 - May 27, 22
Since May 28, 2013

Discharge Summary Medical Oncology

Patient Demography Details :

Name : Vipin Kumar Garg

DOB : 30 MAR, 1957

DOA : 23 SEP, 2025 10:30

Address : HNO. 323 FF, SEC P PANCHKULA HR, Panchkula Industrial Area Phase 1, HARYANA, 134113

Patient ID : MHLI.435714

Age/Gender : 68 Years/MALE

Ward : MHLI-LB-ONCOCARE

IP No. : 252053

Primary Consultant 1 : Sachin Gupta

Primary Consultant 2 : Krutika Kurhade

Mobile No. : 9815311118

Date and Time of Discharge: 23 SEP, 2025 14:09

Diagnosis:

Multiple Myeloma

C- R- A- B+

ISS-II

RISS-II

Received 4 cycles with Inj Bortezomib /Inj Daratumumab/ Inj Cyclophosphamide till 03.10.2022

Relapse in Feb 24

Post 6 cycles with Inj Carfilzomib + Inj Daratumumab + Inj Zoledronic and Tab Pomalipomide till 04.09.2024.

Acute kidney injury in sept 2024.

Patient was on Maintenance Daratumumab + Tab Pomalidomide from Jan 2025.

Treatment Plan:

Plan:-

Inj Daratumumab 1200 mg D1Q monthly

Inj Denosumab 120 mg s/c stat3 monthly

Cap Pomalidomide 2 mg 1 cap 3 weeks on & 1 week off

Brief Summary :

Mr. Vipin Kumar Garg, 65 years gentleman was apparently alright till May end 2022 when he actually started having complaints of pain in back region and right side of chest for which he went to PGI, Chandigarh where he was evaluated and investigated.

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May 28
Sim

Item Name	Date	Qty	Unit	Unit Price	Amount(Rs.)
5 DARZALEX FASPRO 1800MG INJ (Batch No./ Lot No : PKS1F13 / DOE : 31-10-26)	23/9/25	1.0	Vial	227,770.00	227,770.00
Sub total :					227,770.00
Sub Total :					227,777.93
Medical Consumable					
6 GLOVES-STERILE SURGICAL 7.0 - NULIFE (Batch No./ Lot No : TSPSMB3G25 / DOE : 30-06-30)	23/9/25	1.0	Pair	110.63	110.63
Sub total :					110.63
Sub Total :					110.63
Other Charges					
7 Admission Charges	23/9/25	1.0		350.00	350.00
8 Medical History Assessment	23/9/25	1.0		350.00	350.00
Sub Total :					700.00
Grand Total :					0.00
Total Deposit					238,188.56
Total Deposit					238,189.00
Net Amount Refunded to Patient :					-0.44

25
YEARS OF
SERVICE AND
EXCELLENCE

Deposit Details :

SNo.	Date	GSTSeriesNo.	Amount
1	23/9/2025		99,000.00
2	23/9/2025		99,000.00
	23/9/2025		40,189.00
Total Deposit Amount :			238,189.00

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www.maxhealthcare.in ID : 252053

GST Bill No. U4540251200800085462

Bill Date : 23/09/2025 13:29



Bill of Supply Inpatient Bill (Detailed)

IPID : 252053
Patient Name : Mr. VIPAN KUMAR GARG
Patient Address : HNO. 323 FF, SEC P PANCHKULA HR Panchkula,
Panchkula Industrial Area Phase 1, Haryana, 134113,
Patient PAN/
Form 60 : MHLI/23-09-2025/252053
Bed : MO-OCDC-14
Primary Doctor : Sachin Gupta

Company :

Company GSTNo. :

Company Address :

Company State :

MaxId : MHLI .0000435714
Admit Date&Time : 23/09/2025 10:30
BillNo : MHIC293468
Bill Date : 23/09/2025 13:29
Discharge Date&Time : 23/09/2025 13:29
PAN No. : AACCH1518B
GSTINNo. : 03AACCH1518B1ZG
GST Bill No. : HM25ID0000008634
SACCODE : 999311
Place Of Supply : Punjab-(03)

Sl. Item Name No.	Date	Qty	Unit	Unit Price	Amount(Rs.)
1 Chemotherapy Day Care Major (Medica Onco) (100%) (Day Care) (Package)	23/9/25 10:36 -23/09/25 13:29	1.0		9,600.00 #	9,600.00
Sub Total : 9,600.00					9,600.00

Drugs

2 AVIL 25MG TAB (1x15) (Batch No./ Lot No : 5P0025A / DOE : 30-12-27)	23/9/25	2.0	Tablets	0.83	1.66
Sub total : 1.66					1.66

CO

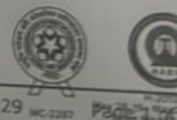
AURADEX 4MG TAB (1X10) (Batch No./ Lot No : T5-279. / DOE : 30-06-27)	23/9/25	1.0	Tablets	4.13	4.13
CALPOL 650MG + TAB(1X15) (Batch No./ Lot No : EC25115 / DOE : 30-06-27)	23/9/25	1.0	Tablets	2.14	2.14
Sub total : 6.27					6.27

Max Super Speciality Hospital, Mohali
(A unit of Hometrail Buildtech Pvt. Ltd.)
Near Civil Hospital, Phase - 6, Mohali,
Punjab - 160 055
For medical service queries or appointments,
Call: +91-172 521 2000

Hometrail Buildtech Pvt. Ltd.
Regd. Office: N - 110, Panchsheel Park,
New Delhi - 110 017
T: +91-70424 01894
E: secretarialhbp@maxhealthcare.com

IPID : 252053

GST Bill No. U45H0251200800663462 Bill Date : 23/09/2025 13:29





Bill of Supply Inpatient Bill(Summary)

Patient Name : 252053
: Mr. VIPAN KUMAR GARG
PatientAddress : HNO. 323 FF, SEC P PANCHKULA HR
Panchkula, Panchkula Industrial Area Phase 1,
Haryana, 134113, India
Patient PAN /Form 60 : MHLI/23-09-2025/252053
Bed : MO-OCDC-14
Doctor : Sachin Gupta
MaxID : MHLI .0000435714
BillNo : MHIC293468
Bill Date : 23/09/2025 13:29
Admit Date&Time : 23/09/2025 10:30
Discharge Date&Time : 23/09/2025 13:29
PAN No. : AACCH1518B
GSTIN No. : 03AACCH1518B1ZG
GST Bill No. : HM25ID0000008634
SACCODE : 999311
Place Of Supply : Punjab-(03)
CompGSTNNo. :

Sl.No.	Service Name	Amount(Rs)
1	Chemotherapy Day Care Major (Medica Onco) (100%) (Day Care) (Package)	9
2	Drugs	22
3	Medical Consumable	
4	Other Charges	
5	Room Rent	

Grand Total :

2

Bill Amount :
Deposit :

Net Amount refunded to patient :

Deposit Details :

No.	Date	GSTSeriesNo.	Amount
	23/9/2025	HM25IF0000017997	99,000.00
	23/9/2025	HM25IF0000017998; UPI I	99,000.00
	23/9/2025	HM25IF0000018003	40,189.00
Total Deposit Amount :			238,189.00

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Hometrail Buildtech Pvt. Ltd.

Regd. Office: N - 110, Panchsheel Park,
New Delhi - 110 017

GST Bill No. : HM25ID0000008634

E: secretarialhbp1@maxhealthcare.com

(CIN: U45400DL2008PTC176962)

Bill Date : 23/09/2025 13:29

Settlement Receipt

MaxId : MHLI .0000435714
IPNO : 252053
Patient Name : Mr. VIPAN KUMAR GARG
Doctor : Sachin Gupta
Bed : MO-OCDC-14
Patient Address : HNO. 323 FF, SEC P PANCHKULA HR Panchkula,
Panchkula Industrial Area Phase 1, Haryana,

GST Receipt No. : HM25IH0000010022
Receipt No. : MHRI208269
Receipt Date : 23/09/2025 1:30 pm
AdmitDateTime : 23/09/2025 10:30 am
GST No. : 03AACCH1518B1ZG
State(Code) : Punjab-(03)

SL.No.	Service Code	Service Name	Mode of payment	GST Rate %	GST Amt	Amount
1	999311	Inpatient services	Cash	0.00	0.00	

Received Rupees only With thanks From Mr. VIPAN KUMAR GARG

Net Amount (Rs) :

Total Bill Amount(Rs) : 238,189.00

Company credit limit(Rs) : 0.00

Amount paid by patient(Rs) : 238,189.00

Deepak Kumar
Operator

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(CIN: U45400DL2008PTC176962)